



GOVERNMENT OF SIERRA LEONE

MINISTRY OF HEALTH AND SANITATION

NATIONAL MALARIA CONTROL POLICY DOCUMENT

REVISED VERSION 2024

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Finally, we thank the communities, civil society organizations, and the private sector for their continued engagement and support in malaria programming. The successful implementation of this policy depends on the collective efforts of all stakeholders, and we look forward to continued collaboration in achieving a malaria-free Sierra Leone.

1. INTRODUCTION

Malaria remains a significant global health challenge, with about 3.2 billion people—almost 40% of the world's population—at risk. In 2023, there were approximately 247 million cases worldwide, with 619,000 deaths. Sub-Saharan Africa continues to bear the highest burden of malaria, accounting for 95% of global cases and 96% of deaths, most of which occur in children under five years old.

In Sierra Leone, malaria remains endemic with stable and year-round transmission. The disease is the leading cause of morbidity and mortality, especially among children under five. Data from 2023 show that malaria caused about 40.3% of outpatient consultations across all age groups and 47% among children under five. Malaria also contributed to 37.6% of hospitalizations, with a case fatality rate of 17.6%. *Plasmodium falciparum* is the predominant parasite species, responsible for over 90% of cases, transmitted mainly by *Anopheles gambiae* and *Anopheles funestus* mosquitoes.

Efforts to combat malaria in Sierra Leone align with global goals, including the WHO Global Technical Strategy for Malaria 2016–2030. However, challenges such as health systems disruptions from past health emergencies, including Ebola and COVID-19, have impacted malaria control efforts, highlighting the need for resilient health systems.

2. BACKGROUND

- 2.1. Malaria is one of the most serious public health problems in Sierra Leone, accounting for over 40.3% of outpatient visits. The most vulnerable groups include under five-year-old children and pregnant women.
- 2.2. The country experiences high endemicity of malaria ranging from mesoendemic to hyper/holoendemic. Malaria continues to be a major health issue in Sierra Leone, with a national prevalence rate of 22% as of 2024.
- 2.3. *Plasmodium falciparum* which also causes severe malaria accounts for more than 90% of all malaria infections in the country. However, there are occasional occurrences of mixed infections with *P. malariae* and *P. Ovale*.
- 2.4. The main vectors are *Anopheles gambiae s.l.*, *Anopheles funestus* and *Anopheles melas*.
- 2.5. Transmission occurs throughout the year with peaks at the beginning and the end of the rainy season (April and October respectively).

3. PURPOSE OF POLICY DOCUMENT

This policy outlines the framework for malaria control activities in Sierra Leone, aiming to reduce the disease burden and get on track for malaria elimination. It is designed to be flexible, with provisions for

revisions based on new data or evolving needs, ensuring ongoing effectiveness in malaria prevention and disease management. The policy will be operationalized through relevant regulations, strategic documents, and guidelines.

4. STRATEGIES AND INTERVENTIONS

The Ministry of Health and Sanitation, in line with the WHO Global Technical Strategy, and the Roll Back Malaria initiative has adopted the following Strategies and Interventions:

- Case Management
- Multiple Preventive Methods
- Capacity Building
- Advocacy, Information, Education, Communication and Social Mobilization
- Operational Research
- Surveillance, Monitoring and Evaluation
- Partnership Strengthening and Programme Management

5. CASE MANAGEMENT

Early diagnosis and provision of prompt and effective treatment is a major strategy to reduce the burden of malaria. Access to the highest quality of care is critical to reducing morbidity and preventing deaths. Rapid malaria diagnosis, with prompt, effective treatment, is delivered through all healthcare sectors, including public, private, and community channels.

5.1. Malaria diagnosis

Malaria diagnosis is an essential step for the management of malaria.

- 5.1.1 Prompt parasitological confirmation by microscopy or alternatively by Rapid Diagnostic Test (RDT) shall be done in all patients suspected of malaria before treatment is started.
- 5.1.2 Microscopy shall be used at tertiary, secondary facilities and Community Health Centers.
- 5.1.3 RDTs shall be used at all levels of care as an alternative where microscopy may not be possible or feasible, including at community level. The necessary systems and measures shall be established to facilitate availability and use of RDTs within the community health services package.
- 5.1.4 Quality assurance of malaria diagnostics shall be included within the national laboratory quality management system.

5.2. Treatment of Malaria

Prompt malaria treatment is essential to reduce morbidity and mortality.

- 5.2.1. Treatment of uncomplicated malaria shall be the same at all levels of the health care delivery system including at community level.
- 5.2.2. The medicine of choice for treatment of uncomplicated malaria shall be an effective antimalarial medicine based on WHO guidelines considering sensitivity of parasites in the country.
- 5.2.3. The use of oral artemisinin-based **monotherapy** for the treatment of uncomplicated malaria is prohibited.
- 5.2.4. The recommended antimalarial medication for uncomplicated malaria shall be classified as Over the Counter (OTC) medicines permissible to be dispensed at all levels to ensure availability to the general public.
- 5.2.5. All cases diagnosed as severe malaria shall be managed at the appropriate level of care as per guidelines for the case management of malaria in Sierra Leone.
- 5.2.6. Parenteral medication shall be deployed for the initial treatment of severe malaria. This medicine should be strictly reserved for treatment of severe malaria at health facility levels where parenteral medicine administration can be properly supervised.

- 5.2.7. Pre –referral treatment for severe malaria shall be provided before referral in situations where complete treatment of severe malaria is not feasible including at the community level.
- 5.2.8. Detailed specific medicines and guidance for the management of malaria shall be provided through regularly updated *National Malaria Case Management Guidelines*.
- 5.2.9. Antimalarial drug efficacy shall be assessed periodically through regular therapeutic efficacy studies (TES).
- 5.2.10. Policies and structures to facilitated rational deployment and use of antimalarial to mitigate the development and spread of antimalarial resistance shall be ensured.
- 5.2.11. All anti-malarial medicines in use in Sierra-Leone must be registered with the Pharmacy Board of Sierra Leone and shall be subjected to quality control analysis.

5.3. Supply of antimalarials medicine

One of the major goals of the Ministry of Health (MoH) is to ensure timely and adequate supply of antimalarial commodities to its populace.

- 5.3.1. Local manufacturers, importation, distribution, sale and supply of anti- malarial medicines shall be encouraged to ensure regular availability and affordability.
- 5.3.2. Recommended antimalarial medicines for the treatment of uncomplicated and severe malaria shall be available at all health care facilities through the Ministry of Health supply chain mechanisms.
- 5.3.3. Antimalarial therapy shall be provided **free of charge** by Government at public health facilities, and selected NGOs and private sector Health Facilities.

6. MULTIPLE PREVENTION

The Ministry of Health has endorsed the use of appropriate malaria prevention strategies in Sierra Leone to reduce the incidence of malaria. These strategies include vector control, preventive chemotherapy, malaria vaccine, and personal protective measures.

6.1. Vector control measures

Vector control is an essential element of controlling the malaria vector that causes the disease. Vector control interventions are undertaken with the view of preventing, reducing and interrupting disease transmission.

Equitable access to effective malaria vector control interventions tailored to each transmission setting is critical to achieving malaria control and elimination goals. The vector control interventions currently recommended by WHO include:

- a. Insecticide Treated Nets (ITNs)

- b. Indoor Residual Spraying (IRS)
- c. Larval Source Management
- d. Entomological Surveillance

Various methods including campaigns, continuous distributions through integration to other primary healthcare services will be used to ensure equity and coverage.

6.2. Preventive Chemotherapy

Preventive chemotherapy entails the intermittent use of full treatment courses of anti-malaria medicine either alone or in combination in defined vulnerable populations to prevent malaria infection. The currently recommended chemopreventive intervention by WHO includes:

- Intermittent Preventive Treatment for pregnant women (IPTp).
- Perennial Malaria Chemoprevention (PMC)
- Seasonal Malaria Chemoprevention (SMC)
- Intermittent Preventive Treatment in School children (IPT-Sc)
- Post Discharge Malaria Chemoprevention (PDMC)

The choice of chemoprevention interventions will be guided by the epidemiology of malaria in Sierra Leone.

6.3. Malaria Vaccine

The use of recommended malaria vaccine targeting the appropriate population will be deployed as part of the malaria control interventions in Sierra Leone.

6.4. Personal Protection

While protection at individual level can reduce the chance of the individual acquiring the disease, it plays only a limited role in vector control. Some personal protective measures include the use of insect repellent and protective clothing, among others.

6.5. Environmental Sanitation

Environmental Sanitation is the cornerstone strategy for vector control. Communities will be mobilized and sensitized to improve drainage systems and environmental cleanliness to reduce mosquito-human contact and proliferation of the vector.

7. CAPACITY BUILDING

Health Care workers, policy makers, line Ministries, relevant training institutions, Health Care Administrators in the public and private sector, and the general public shall be well informed about the revised malaria policy.

7.1 The National Malaria Control Programme and partners shall lead the development and updating of all standard guidelines and training materials and ensure all trainings align with the materials.

7.2 The National Malaria Control Programme will continue to provide oversight of trainings, supervision and mentorship for all health care workers (public and private).

8. ADVOCACY / IEC / SBC

Advocacy/IEC/SBC are critical components in the implementation of National Malaria Control Programme strategies.

8.1 The National Malaria Control Programme and the Health Education Program of the Ministry of Health, in collaboration with additional partners, are the designated authorities charged with the responsibility of leading the development, production and dissemination of IEC/SBC materials for malaria prevention and control through appropriate channels.

8.2 The National Malaria Control Programme will regularly update IEC/SBC materials.

9. OPERATIONAL RESEARCH

Operational research is vital for context adaptation of strategies and tools leading to improved malaria programming, policies, guidelines, and approaches.

9.1 The Ministry of Health will ensure the relevant capacity (institutional, human, and financial) necessary to conduct operational research into malaria interventions and implementation.

9.2 The NMCP, in collaboration with relevant stakeholders, shall periodically develop and review a comprehensive Malaria research agenda.

9.3 All research proposals must be approved by the Sierra Leone Ethics and Scientific Research Committee prior to conducting research.

10. MONITORING AND EVALUATION

The NMCP is responsible and accountable for monitoring and evaluating all malaria prevention, care and control activities. They will additionally prioritize appropriate use of interventions, and monitor progress, and resource allocation.

10.1 The National Malaria Policy will be monitored through periodic review of the malaria strategy based on a monitoring and evaluation plan with defined indicators and methods for assessing the progress of malaria control and elimination.

10.2 The Ministry of Health will strengthen the existing surveillance system for monitoring malaria as part of an Integrated Disease Surveillance and Response System.

10.3 The pharmacovigilance of antimalarials shall be undertaken within the context of the national pharmacovigilance system, run by the Pharmacy Board.

10.4 The directorate of policy planning and information (DPPI) should make the national malaria data collection and sharing platform available to private sectors and encourage them to input their data into it.

11. THE INSTITUTIONAL FRAMEWORK OF THE NATIONAL MALARIA CONTROL PROGRAMME

11.1 The NMCP is responsible for all technical matters relating to malaria in the Ministry of Health. All malaria programming and implementation will be adapted to the health system and structure delivery of the MoH.

12. PRIVATE SECTOR

The private sector plays a critical role in malaria programming and implementation. The Ministry of Health will facilitate collaboration with all relevant private sector stakeholders in the fight against malaria.

12.1 The Ministry of Health, in collaboration with other stakeholders, will lead the development and operationalization of a functional private sector engagement strategy.

13. PARTNERSHIPS

Developing strong partnerships with a wide range of organizations is integral to progressing the country's malaria agenda.

13.1 The NMCP, with the Ministry of Health, will take the lead in facilitating and ensuring a focus on developing functional partnerships using the principles of the three ones: one program, one plan, and one strategy.

14. RESOURCE MOBILIZATION

Identifying and mobilizing sufficient resources enables planned activities to be actioned – from prevention and treatment to monitoring and evaluation – and overall impact of malaria control efforts to be strengthened.

14.1 The NMCP will take the lead to ensure adequate resourcing for the implementation of the National Strategic Plans.

14.2 The NMCP shall ensure adequate costing of national strategic plans and develop an appropriate resource mobilization strategy.

15. INNOVATION AND NEW RESOURCES

Innovation and evolving malaria resources help address emerging challenges from biological threats, and malaria in particular.

15.1 The NMCP and Ministry of Health will promote the use of existing tools and the development and deployment of new tools and innovations, including digital solutions, as they emerge and are approved.

15.2 The Ministry of Health will create an enabling environment that promotes research, innovation, and progress.

16. POLICY REVIEW AND UPDATES

This policy aligns with the WHO Global Malaria Frameworks. It will be reviewed if there is a significant shift in malaria eco-epidemiological and entomological landscape at the country and global level to the extent that it affects Sierra Leone or the region, or as otherwise necessary

ANNEXES

AL - Artemether and Lumefantrine

AS + AQ - Artesunate and Amodiaquine

BCC - Behavioral Change Communication

CH/EPI - Child Health / Expanded Programme on Immunization

CHW - Community Health Worker

DHMT - District Health Management Teams

IEC - Information, Education, and Communication

IMNCI - Integrated Management of Newborn and Childhood Illness

IPT - Intermittent Preventive Treatment

IPTp - Intermittent Preventive Treatment for pregnant women

IPT-Sc - Intermittent Preventive Treatment in School Children

IRS - Indoor Residual Spraying

ITN - Insecticide-Treated Nets

LLIN - Long-Lasting Insecticidal Nets

M&E - Monitoring and Evaluation

MIS - Malaria Indicator Survey

NGO - Non-Governmental Organization

NMCP - National Malaria Control Programme

PDMC - Post-Discharge Malaria Chemoprevention

PMC - Perennial Malaria Chemoprevention

PMTCT - Prevention of Mother-to-Child Transmission

RBM - Roll Back Malaria

RDT - Rapid Diagnostic Test

RH/FP - Reproductive Health / Family Planning

SBC - Social and Behavior Change

SP - Sulfadoxine-Pyrimethamine

TES - Therapeutic Efficacy Studies

UNICEF - United Nations Children's Fund

VDC - Village Development Committees

WHO - World Health Organization

Annex 2: References

National Strategic Plan for Malaria Control in Sierra Leone – September 2016-2020

Revised Guidelines for the Case Management of Malaria in Sierra Leone – September 2015

National Guidelines and Strategies for Malaria Prevention and Control During Pregnancy in Sierra Leone – April 2018

Policy Guidelines on Insecticide-Treated Nets (ITNs) – November 2018

Policy Guidelines for Integrated Vector Management – May 2019

Malaria Indicator Survey (MIS) – June 2016

Therapeutic Efficacy Studies for Antimalarial Drugs – Most recent study (2024, where applicable)

WHO Global Technical Strategy for Malaria 2016–2030 – World Health Organization

Sierra Leone Integrated Disease Surveillance and Response Strategy – Ministry of Health and Sanitation (latest edition)

National Malaria Case Management Guidelines – Revised edition (year to be updated per latest document).